Vorsorge

Her wishes exist. Can the care team use them?

Advance-care planning and document handoff

Erika Berger is a synthetic case. All demo patient data are synthetic.

The preparation gap

About 1 in 2

adults aged 45–90 reported none of three advance-care documents.

Our question: once wishes exist, can a care team locate the relevant version and passage?

DEAS 2023, ages 45–90, self-report. Advance directive, power of attorney, guardianship directive.
Source: Sozialbericht 2024, chapter 2.7, pp. 100–101.

Two connected jobs

SEPARATE LOCAL PREVIEW

Prepare wishes

Guided steps help a person start and review a draft.

AT THE POINT OF CARE

Use the source

Share an existing version.
Confirm the case and receipt.
Open the original passage.

Current scope: preparation and existing-document handoff remain separate flows.
Today’s live demo follows the existing-document handoff.

Live demo

One version.
One confirmed case.
One source passage.

1. Share version 2
2. Receive and confirm
3. Open the original PDF
4. Check the receipt

2 minutes 30 seconds
Erika Berger, synthetic demonstration

The source supports review. A clinical professional remains responsible for interpretation.

AI and technical scope

IMPLEMENTED NOW

Guided rules and explicit handoff
Prepared links to source passages

NEXT HYPOTHESIS

AI proposes extracted passages.
A person checks the source.

No runtime AI model in this prototype.
No live hospital-system integration.

Commercial hypothesis

Hospitals fund workflow integration and ongoing service.

FIRST USER	Clinical teams handling incoming directives
VALIDATION	Buyer interviews and a measured workflow pilot

Hypothesis only. We make no validated customer, pricing or cost-savings claim.

What this submission includes

BUILT DURING THE EVENT

Synthetic personal and clinical flows
Document version and receipt trail
PDF source passage view

PRE-EXISTING INPUTS

General preparation and prompts
Official templates and published research
Development tools and libraries

Development used AI assistance. The prototype uses synthetic cases.

Demo code snapshot: 6ec45fe

A clinical partner for the next test

Can staff find the correct passage
and keep the right version and recipient?

One clinical workflow partner
One integration partner

Talk to our team after the pitch

Sources and market context

A1

Advance-care preparation

DEAS 2023: roughly half of people aged 45–90 reported none of three advance-care documents. The population and self-report basis matter.

Existing infrastructure

ZVR supports electronic document storage through approved institutions from 1 October 2026. Our proposed role is the clinical workflow.

Institutional context

Germany had 1,841 hospitals in 2024. This is an institutional count, not a market-size estimate or a count of buyers.

Prototype boundaries

REPOSITORY CAPABILITIES

Synthetic document handoff
Explicit patient and case confirmation
Prepared PDF source links
Document version and receipt trail

STILL TO VALIDATE

Runtime AI extraction
Live NFC and hospital integration
Production access controls
New planning draft in the clinic
Clinical and legal suitability

Historical repository report at 6ec45fe: 20 scenario tests passed.
That report does not establish clinical effectiveness or deployment readiness.

Questions we are ready to test

Who pays?	Hospital or provider group, subject to buyer interviews.
Why alongside ePA and ZVR?	Case confirmation and access to the original source. Integration remains unproven.
What is success?	Compare time to find a passage, version accuracy and recipient accuracy with current practice.
What does AI do today?	The app uses rules. AI assisted development. Runtime extraction is future work.